



FAIRBANKS MEDICAL CENTRE LIMITED

Your Health, Our Mission

Kyebando-Kisalosallo, Tirupati Road (opposite the Northern Bypass roundabout), Kampala, Uganda

INVESTMENT PROPOSAL

Maternity and Inpatient Unit with Obstetric Theatre

Prepared for Mr. Alex Kivumbi · 7 September 2026

Ref: FBMC/INV/2026/002

7 September 2026

IN SHORT

FairBanks is asking for UGX 192-265M to complete and commission a maternity and inpatient unit with its own obstetric theatre, built in two phases. Phase one costs UGX 75-100M and can begin immediately. Phase two, the theatre, can be committed once phase one is open and reporting.

UGX 192-265M

COMPLETE PROGRAMME

UGX 75-100M

PHASE ONE, STARTS NOW

14-18 months

PHASE ONE CAPITAL RECOVERY

1. Why maternity, and why now

FairBanks Medical Centre operates in Kyebando-Kisalosallo, on the Northern Bypass. The centre is licensed and running: outpatient consultations, laboratory, pharmacy, X-ray, specialist clinics, physiotherapy and a 24-hour emergency service, together producing between UGX 8M and UGX 12M a month. We accept CIC and AAR, with Jubilee and Mutual in process.

Obstetrics and gynaecology is already one of our clinics. What we do not have is anywhere to admit the women who use it. An expectant mother can have her antenatal care with us, her scans with us and her medicines from our pharmacy, and then deliver somewhere else, because at the point where she needs a bed we have nothing to offer her. Every one of those deliveries is revenue we have already paid to acquire and then handed to another facility.

The maternity and inpatient unit closes that gap. It is also the only expansion open to us that creates a genuinely new service line rather than adding capacity to one we already run.

2. What we are asking you to fund

The work splits cleanly into two phases. Phase one produces a functioning maternity and inpatient ward. Phase two adds the operating theatre and neonatal capacity that let us keep emergencies in the building.

PHASE ONE: MATERNITY AND INPATIENT WARD

ITEM	WHAT IT COVERS	ESTIMATE
Ward completion and fit-out	Partitioning, flooring, plumbing, sluice room, washrooms, lighting and power points for the delivery room, postnatal room and inpatient bays	UGX 22-30M
Delivery and ward equipment	Two delivery beds, ten patient beds and lockers, delivery and episiotomy sets, examination couches, vital-signs monitors, suction units, a resuscitaire and oxygen concentrators	UGX 28-36M
Obstetric ultrasound	Mindray DP-10 with printer and trolley. Quoted at UGX 11,000,000 by Canon Healthcare Systems Ltd, proforma 1896 of 13 April 2026	UGX 11M
Staffing and launch	Recruitment and first-quarter cost of UNMC-registered midwives and ward nursing cover, orientation, licence variation and inspection	UGX 10-14M
Opening working capital	Oxytocin and essential obstetric medicines, sutures, gloves, cord clamps, linen, laundry and utilities for the first three months	UGX 6-9M
Phase one total		UGX 77-100M

PHASE TWO: OBSTETRIC THEATRE AND NEONATAL CARE

ITEM	WHAT IT COVERS	ESTIMATE
Theatre construction and services	Theatre, scrub area, sterile store and sluice; sealed walls and flooring; ventilation; theatre lighting; dedicated power with UPS and generator backup	UGX 30-45M
Anaesthesia and surgical equipment	Anaesthetic machine with ventilator, operating table, theatre lights, diathermy unit, theatre and recovery monitors, theatre suction	UGX 45-60M
Instruments and sterilisation	Caesarean and gynaecological instrument sets, autoclave, instrument trolleys and sterile storage	UGX 14-20M
Neonatal capacity	Incubator, phototherapy unit and neonatal resuscitation equipment	UGX 12-18M
Theatre staffing and commissioning	Obstetric and anaesthetic cover, theatre nurse, commissioning, clinical protocols, blood-supply arrangement and licensing for surgical services	UGX 14-22M
Phase two total		UGX 115-165M

COMPLETE PROGRAMME

UGX 192-265M. Every figure above except the scanner is our own estimate, prepared before going to suppliers. They are ranges because that is honestly what we know today. No funds should be released against an estimate: each line will be quoted and returned to you as a signed one-page budget first.

3. What the unit should earn

The figures below are FairBanks' planning assumptions, not results. They are set out in full so you can argue with them, which is the point of showing the arithmetic rather than the conclusion.

PHASE ONE AT STEADY UTILISATION

SERVICE	PER MONTH	PRICE (UGX)	REVENUE (UGX)
Antenatal visits	100	25,000	2,500,000
Normal deliveries	18	450,000	8,100,000
Obstetric ultrasound scans	70	50,000	3,500,000
Inpatient bed-days	70	70,000	4,900,000
Monthly revenue			19,000,000

MONTHLY RUNNING COSTS

COST	UGX
Midwifery and ward nursing: four midwives, a ward in-charge, two support staff	5,500,000
Medicines, sutures and clinical consumables	2,800,000
Utilities, standby power, water and clinical waste disposal	1,200,000
Laundry, cleaning, maintenance and equipment servicing	700,000
Monthly running costs	10,200,000

PHASE ONE MONTHLY CONTRIBUTION

UGX 19,000,000 less UGX 10,200,000 = UGX 8,800,000 a month, once utilisation settles.

WHAT THE THEATRE ADDS

PHASE TWO	UGX
Caesarean sections, 5 a month at UGX 1,700,000	8,500,000
Less obstetric and anaesthetic session fees, 5 cases	(2,500,000)
Less theatre nurse and sterilisation cover	(1,400,000)
Less theatre consumables, drugs and blood handling	(1,300,000)
Less theatre maintenance and equipment servicing	(600,000)
Additional monthly contribution	2,700,000

That takes the combined contribution to UGX 11,500,000 a month. We have not modelled gynaecological use of the theatre, which in practice will carry a share of the load and improve the figure.

At those volumes phase one returns its own capital in about ten months of steady operation. Ten months of steady operation is not ten months from the day you release funds. Construction, recruitment, licensing and the time an antenatal book takes to fill mean the first six months will run below the table above. Our working expectation is capital recovery on phase one in fourteen to eighteen months, and on the full programme including the theatre in two and a half to three years.

FIVE NUMBERS TO REPLACE BEFORE THIS IS FINAL

Current monthly antenatal attendance · the delivery fee we intend to charge · the bed count we are actually building to · our intended bed-day rate · the caesarean fee. Those five drive everything else on this page, and FairBanks' own figures should replace the assumptions above before any agreement is signed.

4. What could go wrong

We would rather set these out ourselves than have you find them.

- **Utilisation.** A delivery room fills from the antenatal book, not from advertising. If antenatal attendance does not grow, the delivery numbers above do not happen. That is exactly why phase one puts the ward and the scanner in first: you get six months of real figures before the larger commitment.
- **Clinical risk.** Obstetrics carries the highest liability of anything we do. The centre already holds professional indemnity and all-risks cover, and both would be reviewed and increased before the unit opens. Protocols, partograph discipline, emergency drills and genuine 24-hour cover are conditions of opening, not refinements to add later.
- **The gap before the theatre.** Until phase two is commissioned, caesareans and complications have to go out. That needs a written referral arrangement with a partner hospital and a transport plan that works at three in the morning, both in place before the first delivery.
- **Regulation.** A maternity unit with a theatre needs the facility licence varied to cover that level of care, UNMC-registered midwives, UMDPC-registered clinicians and compliant clinical waste handling. We would not spend phase two money before the licence position is confirmed in writing.
- **Cost.** Every figure in this document except the scanner is an estimate made before quotation. Firm prices could come back above these ranges, most likely on anaesthesia equipment, which is imported and moves with the exchange rate.
- **Staffing.** Anaesthetic officers and obstetricians are scarce and they move. We propose sessional arrangements in the first year rather than full-time salaries, which keeps the cost variable and the downside smaller.

5. How your money would be protected

- **Separate books.** The unit is accounted for apart from the rest of the centre, with its own revenue and cost ledger from the first day.
- **Release against quotations.** Funds move in tranches against signed supplier quotations and delivery notes, never against the estimates in this document.
- **A one-page report every month.** Capital deployed, antenatal visits, deliveries, theatre cases, revenue, direct costs, monthly contribution and capital recovered to date.
- **Your own eyes.** You may nominate someone to inspect the books, the store and the equipment at any time, without notice.

- **Registered assets.** Capital equipment is asset-tagged, insured and listed, and can be named in the investment agreement as security.

FairBanks Medical Centre Limited is a registered Ugandan company, TIN 1053370026. The centre holds a current operating licence, a National Drug Authority licence for the pharmacy, a certificate of suitability of premises, NSSF registration, and professional indemnity and all-risks insurance. Copies of all of these are available for your review before you commit anything.

6. How the investment could be structured

- **A project loan.** You lend a fixed amount for a fixed term at an agreed rate, repaid from the unit's contribution. Simplest to document, and your return does not depend on how well we run the place.
- **Revenue share until recovery, plus a premium.** An agreed share of the unit's monthly contribution comes to you until your capital and an agreed premium are repaid, after which the arrangement ends. Your return tracks performance, and repayment only starts once there is something to share.
- **Equity.** A shareholding in the unit or in the company, giving you a permanent share of the upside and a voice in the decisions.

Our own preference is the second. It ties what you earn to how well we run the unit, it does not saddle the centre with fixed repayments during the months when utilisation is still building, and it has a defined end. That said, we are open on this. The structure should be the one that suits you.

7. What we would like to agree

1. You tell us the level of investment you would be comfortable considering, and which structure you would prefer.
2. We put every line in section 2 to suppliers and come back within three weeks with a priced one-page budget, a tranche schedule and a cash-flow projection.
3. We agree the return formula, the reporting format and the release conditions in writing.
4. The first tranche is released and phase one begins.

We would be grateful for a meeting at the centre within the next two weeks, so you can see the space before you decide anything.

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